

Major Depressive Episode Treatment Disparities Among Black and Latino Youths

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A. RESEARCH QUESTION

Our research question asks: How do perceptions of mental-health stigma within Black and Latino communities shape help-seeking behavior, access to care, and sustained engagement with treatment among U.S. adolescents aged 12–20 experiencing Major Depressive Episodes (MDEs)? We chose to focus on this question because Black and Latino adolescents experience significant disparities in mental-health treatment, including lower rates of service use, delayed entry into care, and higher likelihood of relying on crisis or emergency services. Although prior work documents these disparities, much less is known about the mechanisms driving them, particularly how culturally rooted stigma, fear of judgment, concerns about labeling, and mistrust of providers shape whether youth seek help or remain engaged in treatment. MDEs represent a critical point of vulnerability, where untreated symptoms can escalate into academic decline, family strain, and increased risk of self-harm. By examining how stigma influences the full pathway from recognizing symptoms to continuing care, this project aims to address an important gap and help inform the development of culturally responsive, youth-centered interventions that reduce barriers and support sustained engagement for Black and Latino teens.

B. LITERATURE REVIEW

Black and Latino adolescents in the United States experience comparable or higher rates of Major Depressive Episodes (MDEs) than their White peers, yet remain significantly less likely to access or remain engaged in mental-health treatment. Structural inequities, cultural stigma, mistrust, and disparities in digital access collectively contribute to delayed help-seeking and lower treatment continuity. Given these intersecting barriers, the research question guiding this paper - How do perceptions of mental-health stigma within Black and Latino communities shape help-seeking behavior, access to care, and sustained engagement with treatment among adolescents aged 12–20? - addresses a persistent health disparity with profound implications for long-term mental-health outcomes. The following literature review synthesizes findings across

13 peer-reviewed articles to explore the problem space, identify remaining gaps, and examine both analog and digital attempts to address this inequity.

B1. Problem Space

B1.1 Disparities in Access, Treatment Initiation, and Engagement

Across large national datasets, Black and Latino adolescents consistently show lower rates of mental-health service use than their White peers. Flores et al. (2023) found that although MDE prevalence is high across racial/ethnic groups, minority adolescents especially Black youth are significantly less likely to receive therapy or pharmacological treatment. Similarly, Ma et al. (2025) reported that only about one-fifth of Black adolescents accessed any mental-health service in the past year, compared with nearly one-third of White adolescents. Latino adolescents also exhibited lower rates of outpatient, school-based, and telemental-health service utilization, indicating a multilayered inequity in access to care. Structural inequities reinforce these disparities. Rodgers et al. (2021) demonstrated that even after major policy reforms, spending and access gaps persisted across 2010–2017, disproportionately limiting minority adolescents' use of specialty mental-health services. Hoffmann et al. (2022) similarly found that pediatric mental-health conditions among Black and Latino youth remain systematically undertreated due to shortages of culturally competent providers, insurance barriers, and fragmentation in care systems.

B1.2 Stigma as a Barrier to Help-Seeking and Engagement

Across the literature, stigma consistently emerges as a central mechanism influencing whether Black and Latino adolescents seek and remain in care. Lu et al. (2021), in a systematic review of 32 U.S. studies, identified adolescent internalized stigma, caregiver fear of labeling, cultural norms discouraging emotional disclosure, and ethnic-identity concerns as significant barriers to accessing services. Qualitative research offers deeper insight: Daskalska et al. (2025) found that teens and families often avoid discussing depression due to fears of judgment, shame, and cultural expectations of resilience. Stein et al. (2024) found similar themes in Black youth, where stigma operated through family silence, mistrust of providers, and trauma histories that complicate disclosure. Stigma also shapes treatment engagement. Burrow-Sánchez (2019) found that cultural mismatch and concerns about judgment reduced adherence among Latina/o adolescents in substance-use treatment, suggesting similar patterns may occur in MDE contexts.

Williams et al. (2023) demonstrated that Black adolescent males often disengage prematurely from school-based mental-health resources due to stigma, fear of peer judgment, or beliefs that counseling is not culturally acceptable.

B1.3 Digital and Analog Interventions: Mixed Success

Analog interventions like school counseling, family-centered therapy, and community partnerships play a longstanding role in adolescent mental-health support. However, as Newberry et al. (2024) noted, even in community settings, cultural stigma and limited awareness remained major barriers among Latino families. School-based models show promise, but resource disparities mean that schools serving predominantly Black and Latino students often lack the staff capacity to provide early or sustained intervention (Williams et al., 2023). Digital interventions offer convenience and reach, yet their effectiveness among minority youth is uneven. Emezue et al. (2024) found that telehealth increased access for some Black and Latino adolescents but raised issues related to privacy at home, device availability, and comfort discussing mental health virtually. Lehtimäki et al. (2021) concluded that existing digital mental-health tools rarely incorporate cultural tailoring, limiting their relevance and trustworthiness for marginalized youth. Willis and Neblett (2023) similarly emphasized that mobile mental-health interventions must incorporate cultural values, language, and identity-relevant content if they are to meaningfully serve minority adolescents.

B2. Research Gap

Although substantial research documents disparities in adolescent mental-health service use, the literature still does not fully explain how culturally rooted stigma shapes treatment pathways for Black and Latino youth with Major Depressive Episodes (MDEs). Much of the existing work aggregates all “minority adolescents” into a single category, which obscures important cultural differences between Black and Latino communities. Studies consistently show that stigma, fears of labeling, and mistrust influence help-seeking (Lu et al., 2021; Daskalska et al., 2025), yet few examine how these mechanisms differ based on cultural identity, family norms, or community context. Large-scale quantitative research establishes disparities (Flores et al., 2023; Ma et al., 2025) but rarely investigates *why* minority adolescents delay treatment or disengage early, leaving gaps in our understanding of continuity of care, therapeutic alliance, or cultural safety.

Another gap concerns the specificity of the mental-health condition: most studies combine depression with other forms of distress, making it difficult to understand how stigma interacts with the unique symptoms and risks of MDEs (Hoffmann et al., 2022). Digital interventions, while increasingly common, remain largely generic and unadapted for cultural context. Reviews show that few digital tools incorporate culturally relevant language, identity cues, or family dynamics (Lehtimäki et al., 2021), and emerging work suggests that telehealth environments can feel unsafe or misaligned with cultural norms (Emezue et al., 2024). Even research on culturally adapted mobile tools remains early-stage, with limited evidence on how such interventions might reduce stigma or improve engagement (Willis & Neblett, 2023). Together, these gaps point to the need for research that moves beyond documenting disparities to examining the specific cultural, interpersonal, and stigma-related mechanisms shaping treatment entry and engagement. There is also a need for translating these insights into practical, culturally grounded intervention strategies - particularly digital solutions that are trusted, relevant, and accessible to Black and Latino adolescents with MDEs.

B3. Literature Summary

Across the thirteen studies, a clearer picture emerges of how structural inequities and cultural stigma jointly shape Black and Latino adolescents' mental-health experiences. Large national datasets consistently show that minority youth access mental-health services at significantly lower rates across outpatient, school-based, and digital settings (Flores et al., 2023; Ma et al., 2025). These studies provide strong evidence of disparities but offer limited insight into the cultural or interpersonal mechanisms behind them, often grouping diverse racial/ethnic communities together and masking important differences.

Qualitative research helps explain what quantitative studies cannot. Interviews with adolescents and caregivers reveal that stigma operates not only through individual attitudes but through family norms, expectations around emotional strength, and mistrust of mental-health institutions (Daskalska et al., 2025; Stein et al., 2024). These accounts show how cultural values, fear of judgment, and concerns about labeling contribute to delayed help-seeking and early dropout. Although qualitative studies have smaller samples, they add crucial nuance by illustrating how stigma is embedded in family and community contexts.

Intervention-focused studies reveal why existing solutions only partially address disparities. School-based programs are often the first point of contact, but schools serving Black and Latino youth frequently lack adequate staffing, cultural alignment, or resources to provide sustained support (Williams et al., 2023). Digital tools, while promising, similarly fall short. Reviews show that most apps and telehealth platforms lack cultural tailoring, do not address stigma, and are not designed with minority adolescents' privacy concerns or family dynamics in mind (Lehtimaki et al., 2021; Emezue et al., 2024). Early work on culturally adapted mobile interventions suggests potential (Willis & Neblett, 2023), but remains preliminary and disconnected from the specific challenges of MDE treatment.

Taken together, the literature demonstrates that disparities persist not simply because access is limited but because structural barriers and cultural stigma interact in ways existing interventions rarely address. Quantitative studies document the problem; qualitative studies reveal its cultural roots; and intervention studies show that current solutions are rarely designed with these realities in mind. What is missing is an integrated, culturally informed approach - particularly within digital tools that directly addresses stigma and supports sustained engagement for Black and Latino adolescents experiencing MDEs.

C. KEY INFORMANT INTERVIEW

C1. Key Informant Background

For this project, a key informant interview was conducted with Joshua Tran, a former Clinical Researcher at the UCI Medical Center whose work focused on clinical depression and a current Clinical Researcher for Medical City Dallas. Although his work was not restricted to only Black and Latino youths, he does have direct working experience with all groups. In addition, he has worked with the community and school based intervention programs working with MDE, which made him an appropriate expert who gave us insight into the workings of depression and its treatments within minority adolescents. You can find the interview questions in Appendix G.1.

C2. Interview Learnings

The interview was enlightening to many aspects and it reinforced several of the themes that we had found within the literature review. Joshua emphasized that stigma, family silence, and fear

of judgement are major contributing factors into the barriers of seeking help which most teens are plagued with. This is consistent with what Lu et al. (2021) and Daskalska et al. (2025) had found. In addition, he also wanted to state that although very few teens reach out for help, most of them do want it. They mainly struggle with obstacles like not knowing where to go, mistrust in adults, and mental stigma. Fortunately, Joshua gave a firsthand account of how he had some success in combatting these health disparities. He mentioned the importance of cultural matching saying that adolescents were far more engaged when paired with appropriate bilingual or racially accepting providers, which directly supports findings from Stein et al. (2024) and Willis & Neblett (2023). This lit review showed how crucial culturally responsive care can help build trust. Joshua also further went into detail beyond the literature review explaining about the gap between a teen recognizing they need help and being able to get access to help. He gave many possible aspects that people working in the mental health field can do to improve the conditions that these children are experiencing. However, Joshua specified that the most effective process we can do is directly change the healthcare system we currently have, which is unlikely. To help with our prototype design, he stressed that successful interventions must feel socially familiar, leveraging peer networks and schools, and identify as a safe environment for the kids. Together with the literature review, Joshua helped us confirm insight on the stigma and inequities that the Black and Latino youths go through with MDE and how we can offer better peer support, cultural familiarity, and care for these troubled children.

D. PROTOTYPE

D1. Design Requirements

Our Mentor Chat App's design is driven by eight core requirements aimed at mitigating the cultural and structural barriers that impede Major Depressive Episode (MDE) treatment access and sustained engagement for Black and Latino youth. These requirements prioritize safety, trust, and cultural competence. **Cultural Safety and Relevance:** the design must explicitly facilitate connections between mentees and mentors who share similar cultural backgrounds and lived experiences , fostering an environment of cultural safety to combat stigma. **Trust and Privacy:** ensuring a secure and private messaging space is paramount to building trust and confidentiality , which is critical for overcoming the fear of judgment and stigma associated

with seeking mental health help. **Accessibility:** the app must support multiple languages to ensure Bilingual Accessibility , effectively addressing the diverse needs of Latino youth. **Peer Support Model:** the platform is required to enable secure, one-on-one relationships through Mentorship & Peer Connection between youth and culturally-matched mentors. **Resource Navigation:** the design must integrate Accessible Professional Resources , providing lists of low-cost or bilingual mental health services and a direct pathway for professional contact. **Engagement and Reflection:** key features include Reflective Journaling to support emotion processing and allow for mentor monitoring and progress tracking , alongside mandates for Ease of Use and Positive Social Reinforcement to drive high user engagement and retention

D2. Prototype Features and Innovations

Our solution to address MDE treatment disparities is a Mentor Chat App that incorporates features focusing on peer support, cultural relevance, and safety. The app is designed with distinct interfaces for both mentees and mentors.

The Mentee Version is designed to promote comfort, choice, and anonymity. A central innovation is the swiping style interface, which allows mentees to select a culturally appropriate mentor by viewing detailed bios including cultural background, language, and lived experience. Communication occurs through Secure 1-on-1 Messaging , and the app includes a personal reflection journal (both private and shared) for mood tracking and self-reflection. Furthermore, a resource hub provides access to low-cost or bilingual mental health services and offers a direct pathway for professional guidance.

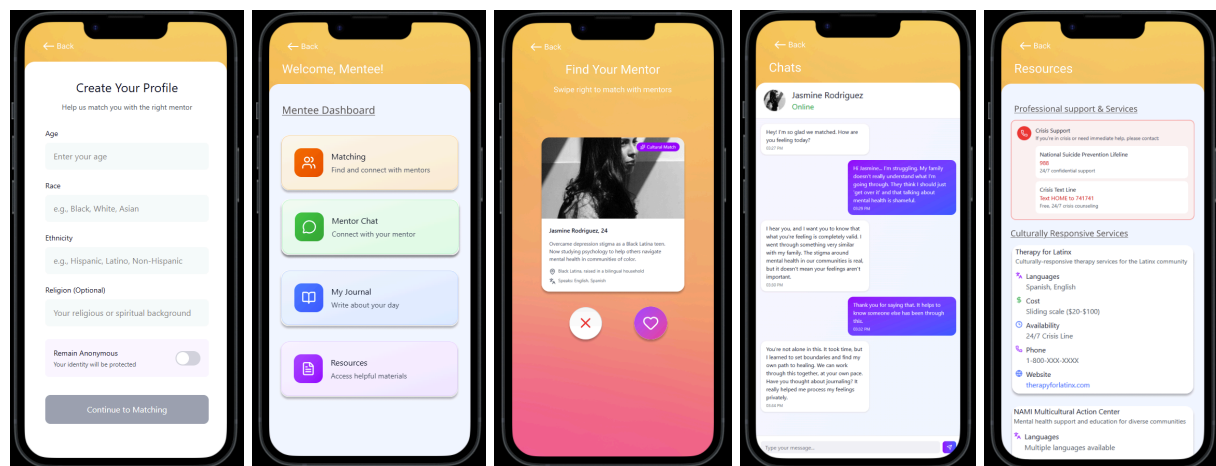


Figure 1: Prototype Mentee Version

The Mentor Version focuses on support, monitoring, and crisis intervention. Mentors create profiles using Cultural Background/Identity Tags and lived experience to ensure culturally relevant matching. A key safety innovation is the Mentor Dashboard, which summarizes mentee check-ins and shared journal entries, enabling mentors to track the mentee's well-being. Also, the dashboard enables Mental Health Escalation, allowing the mentor to immediately identify concerning patterns and escalate at-risk youth to professional services, effectively bridging the gap between peer support and clinical care.

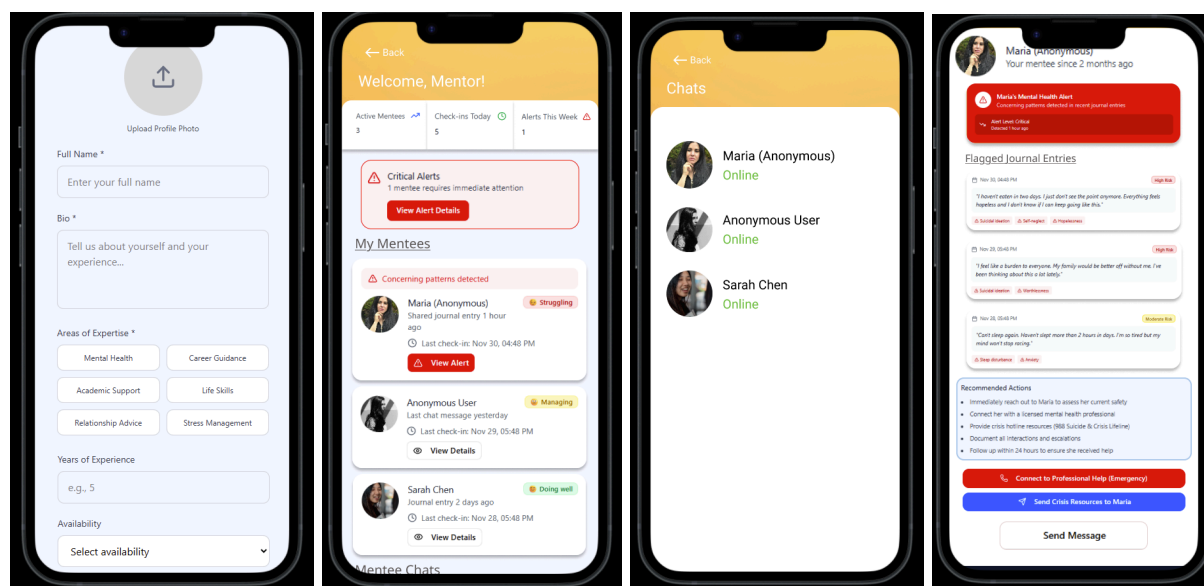


Figure 2: Prototype Mentor Version

D3. Health Disparity Effects

Our Mentor Chat App has the potential to significantly reduce MDE treatment access gaps among Black and Latino kids by addressing the multilayered barriers identified in our research. The app's design addresses stigma, institutional access problems, cultural competence gaps, and the key disconnect between seeking and receiving treatment.

Addressing Cultural Stigma and Structural Barriers: The app's peer mentorship model fundamentally shifts help-seeking from a clinical intervention to a supportive relationship, reducing the "weakness" stereotype that keeps more than half of Black teenagers with MDE from accessing treatment. The app normalizes mental health conversations within the youth's own cultural context by facilitating anonymous contacts with culturally appropriate mentors who share lived experiences. Joshua's interview proved that kids trust their peers more than adults, and our swiping-style mentor selection interface allows them to choose someone who

understands their unique cultural background. The integrated resource hub addresses insurance and cost barriers by connecting users to low-cost, bilingual mental health services, while the bilingual interface ensures that Latino youth are not discouraged by language gaps. Filling the Cultural Competence Gap and Creating Treatment Pathways: According to the KFF survey, people of color face disproportionate challenges in finding providers who understand their experiences, which our matching technology explicitly tackles by linking mentees with mentors who share their racial, ethnic, and experiential backgrounds. Joshua's success story about a Latino student opening up to a multilingual counselor who "looked and spoke like him" supports our design decision to prioritize cultural matching over generic support. Our app bridges the gap between wanting and receiving help through two mechanisms: mentors provide navigation support and encouragement for professional care, and the mentor dashboard's escalation feature ensures that when concerning patterns emerge, mentors can immediately direct at-risk youth to skilled resources. This creates a graduated pathway where peer support serves as the accessible first step that ultimately facilitates clinical intervention, directly addressing the 15.9 percentage-point treatment gap for Latino adolescents compared to White peers.

D4. Ethical Considerations

Deploying our Mentor Chat App in a real-world setting necessitates careful consideration of ethical issues, including child safety, data privacy, peer support boundaries, and equitable access. These factors are crucial to ensuring that the app benefits rather than harms the vulnerable groups it is intended to serve.

Minor Privacy and Mandatory Reporting: The app caters to users aged 12 to 20, creating complicated contradictions between youth autonomy and parental rights. While anonymity encourages young people to seek treatment despite cultural stigma, we must develop explicit standards for required reporting when mentees disclose abuse, self-harm, or suicidal thinking. Joshua underlined that "trust is the foundation," and that consumers must understand how their data is utilized. This necessitates explicit privacy regulations that clearly outline when confidentiality must be breached for the sake of safety. The mentor dashboard's escalation tool

must incorporate automated crisis detection and direct professional referral pathways to ensure mentors are never exclusively liable for life-threatening situations.

Mentor Vetting and Peer Support Boundaries: To ensure mentor quality, background checks and mental health training must be implemented while avoiding prohibitive barriers that could prevent culturally appropriate mentors from serving in target communities. Joshua's observation that culturally responsive care necessitates providers with shared life experiences argues that we should prioritize cultural competence and personal recovery narratives over formal qualifications. To prevent mentees from relying too heavily on untrained individuals and to safeguard mentors from burnout, we must establish explicit boundaries that clarify that mentors provide peer support rather than therapy. The app must protect sensitive mental health information using end-to-end encryption and secure storage, as well as clear data access policies. Joshua raised worries about digital literacy, and the app must work on older devices and lower bandwidth connections to prevent excluding vulnerable youth who do not have constant internet access.

D5. Prototype Link

<https://www.figma.com/proto/oOBFQ43v8TUTDPTftVKT8w/Final-Project-Prototype?node-id=0-1&t=3n4FwSa0syaHbJmh-1>

E. Project Team Member Contributions

Vinay Dara: Conducted a focused literature review examining how cultural stigma impacts help-seeking, access, and treatment engagement for Black and Latino adolescents with Major Depressive Episodes.

Harsh Gandhi: Responsible for conducting literature review research. Also contributed to the design rationale sections and collaborated on prototype development for both mentee and mentor user journeys.

Peter Trinh: Responsible for Key Informant Interview, Key Informant Interview section of paper, and the Figma Prototype

Lu Wang: Responsible for defining the prototype Design Requirements and developing the Prototype Features and Innovations.

F. REFERENCES

1. Identifying barriers to mental health services utilization for Black youth in the United States: A qualitative study. (2024). *SMU Journal of Undergraduate Research*, 9(1), 1–8.
2. Burrow-Sánchez, J. J. (2019). *A randomized trial of culturally accommodated versus standard group treatment for Latina/o adolescents with substance use disorders: Posttreatment through 12-month outcomes*.
3. Daskalska, L., Nelson, D., Smith, J., & Young, S. (2025). *Access to mental health care for Black and Latino teens with anxiety and depression: A qualitative study*. Discover Mental Health.
4. Emezue, C. N., Karnik, N. S., Sabri, B., Anakwe, A., Bishop-Royse, J. C., Dan-Irabor, D., Froilan, A. P., Dunlap, A., Li, Q., & Julion, W. (2024). *Mental telehealth utilization patterns among high school students from racial and ethnic minority backgrounds affected by violence and substance use*.
5. Flores, M. W., Sharp, A., Carson, N. J., et al. (2023). *Estimates of major depressive disorder and treatment among adolescents by race and ethnicity*. JAMA Pediatrics.
6. Hoffmann, J. A., Alegría, M., Alvarez, K., Anosike, A., Shah, P. P., Simon, K. M., & Lee, L. K. (2022). *Disparities in pediatric mental and behavioral health conditions: A state-of-the-art review*.
7. Lehtimäki, S., Martić, J., Wahl, B., Foster, K. T., & Schwalbe, N. (2021). *Evidence on digital mental health interventions for adolescents and young people: Systematic overview*.
8. Lu, W., Todhunter-Reid, A., Mitsdarffer, M. L., Muñoz-Laboy, M., Yoon, A. S., & Xu, L. (2021). *Barriers and facilitators for mental health service use among racial/ethnic minority adolescents: A systematic review of literature*. Frontiers in Public Health.
9. Ma, Y., Ramos, C., Wen, H., & Cummings, J. R. (2025). *Racial and ethnic differences in mental health service use among adolescents*. JAMA Network Open.
10. Newberry, J. A., Gimenez, M. A., Gunturkun, F., Villa, E., Maldonado, M., Gonzalez, D., Garcia, G., Rodriguez Espinosa, P., Hedlin, H., & Kaysen, D. (2024). *Mental health care-seeking and barriers: A cross-sectional study of an urban Latinx community*.

11. Rodgers, C. R. R., Flores, M. W., Bassey, O., Augenblick, J. M., & Cook, B. L. (2021). *Racial/ethnic disparity trends in children's mental health care access and expenditures from 2010–2017: Disparities remain despite sweeping policy reform*. JAACAP.
12. Stein, E., Hutnyan, M., & Myers, N. (2024). *Identifying barriers to mental health services utilization for Black youth in the United States: A qualitative study*. SMU Journal of Undergraduate Research.
13. Williams, E., Lateef, H., Gale, A., Boyd, D., Albrecht, J., Paladino, J., & Koschmann, E. (2023). *Barriers to school-based mental health resource utilization among Black adolescent males*.
14. Willis, H. A., & Neblett, E. W. (2023). *Developing culturally-adapted mobile mental health interventions: A mixed methods approach*.

G. APPENDIX

G.1 Key Informant Interview Questions

1. Many studies show that despite working on the issue, the disparities in mental health treatment haven't improved. What do you think is missing in current research that keeps these improvements from happening?
2. Can you share an example of a time when mental health treatment access worked well for a Black or Latino teen? What made that experience successful?
3. What would a truly culturally responsive and accessible mental health service for Black and Latino youths look like to you?
4. In your experience what are some of the most significant barriers that prevent Black and Latino youths from receiving timely and proper treatment for MDE?
5. Many teens want mental health help, but never actually connect to services. What do you think happens between wanting help and getting it, and how could that process be made easier?
6. What role do you think schools should play in connecting teens to mental health treatment? What would need to change to make school-based mental health services work better?
7. Beyond direct translation, what features of a mobile or digital platform are essential to create a "culturally accommodating" experience for non-English-speaking users?

8. In a digital design context, how can we leverage non-traditional support systems, such as schools and peer networks, to promote the adoption and effective use of mental health interventions?
9. How can we design a mobile interface that is intuitive and accessible for users with varying levels of digital literacy, especially considering potential disparities in digital access among different communities?
10. In your opinion, what could a new service or app do to make mental healthcare more accessible and trustworthy for Black and Latino youths?

“We have not used generative AI to prepare this assignment.”